OCD-05 | For therapists | OCD, ages 8 and over

# Learning from work chosen with the therapist

1 | Purpose, fit and boundaries

## Purpose of this unit

Support discussion of treatment work already adapted and conducted by an OCD-skilled therapist: intended learning, actual events and implications for next steps.

## Choice and adaptation

Choose this sheet following professional assessment and with consent. The person may skip questions, answer aloud, or choose another tool. This is proposed original material for review; it provides no score, diagnosis, or promise of an outcome.

## Boundaries specific to this unit

The form provides no exposure or response-prevention instructions. Do not judge success by immediate distress reduction or forced task completion. Without a documented clinical plan, use questions only for discussion.

## Reusing existing material

Unit 4 records decisions and conditions; this unit focuses on learning rather than repeated confirmation of the plan. General anxiety exercises do not replace specialized OCD work.

## A fictional example for discussion

Fictional example: Dana returns after session work. She describes changing distress and noticing a covert request for certainty. The therapist explores learning and support suitability without grading or adding exercises from the sheet.

> Draft for administrator review. Clinical review and final rights approval are still pending.

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# Working together with the worksheet

2 | A conversation about one situation

## Opening

Choose a situation the person wants to explore. It need not be the hardest situation. Check that the question is understood and the wording fits the person; their account comes before the therapist's interpretation.

## 1 | Question for discussion

What documented attempt did we choose to review now?

## 2 | Question for discussion

What learning question was defined in the session?

## 3 | Question for discussion

What happened and what did I do, including possible covert responses?

## 4 | Question for discussion

What was added to understanding the cycle or functioning beyond a momentary feeling?

## 5 | Question for discussion

What information does the therapist need before adapting the next work?

## Closing

Summarize what became clearer and what remains open. Use the existing SES-S01 and SES-S02 for the session framework. There is no need to repeat an assessment or plan already recorded.

## Possible difficulties

If description becomes precise success checking, summarize aloud. Not attempting, persistent distress or needing clarification calls for exploration, not blame or automatically increased demands.

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# Adapting for four age groups

3 | Age is a starting point, not an assumption about ability

## CH | Children aged 8–11 only

Ages 8–11 only. Read one question and allow a brief or spoken response. Parents add separate observations without deciding the child's thoughts; do not test reporting accuracy. No grades or rewards for absence of fear.

Offer one question at a time with little writing. Adult support must not replace the child's own voice.

## TE | Adolescents, approximately ages 12 to 17

Ages 12–17. Agree what stays private and what is shared with family or school, subject to safeguarding duties. Embarrassing thought content need not be disclosed. Consent to one attempt is not consent to every further attempt.

Agree in advance what will be shared and with whom, and explain the privacy limits in this setting. Do not promise absolute confidentiality.

## AD | Adults

Connect work to chosen goals, family, employment and existing care. Distinguish rituals, practical needs and value-consistent behavior. The learning question is established in treatment, not generated from a blank form.

## OA | Older adults

Preserve adult language, choice and privacy. Assess access, health and desired support without automatically attributing new changes to OCD or assuming reduced ability. Address burden and access without inferring treatment failure.

Offer reading support, time, and conversation according to preference. Choose help from another person together with the participant, respecting their privacy.

> Each age version needs its own review. Approval of one version does not automatically approve the others.

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# Practice, review and next steps

4 | Shared choice and learning from the attempt

## Before offering practice sheet P

The P sheet proposes recording decisions and learning within treatment, not ERP instructions. The therapist determines suitability, action, scope and support. Without an agreed plan, use only for discussion. Do not begin exposure or change response prevention based on this sheet.

## At the next session

Explore learning about the response and ability to act, remaining uncertainty and plan elements needing the therapist's decision. Follow-up recording is not a daily obligation.

## Professional and technical records

Each version has a separate review record covering age suitability, originality, rights, and fictional filling. Reviewer and date fields remain blank until assigned. File checks or a code merge are not clinical approval. Clinical information stays only in the authorized setting, outside the code repository.

## Background references

The following links were used for background only. Questions and adaptations were written afresh for the expansion plan; no questionnaire, scale, or protocol was copied. Adult sources do not validate the child versions. This material makes no claim about session count or treatment effectiveness.

## Treatment-manual mapping status

Mapping against a full OCD manual and professional approval remain pending. This is original support material for review, not an approved ERP manual or its replacement.

www.nice.org.uk/guidance/cg31/chapter/Recommendations
iocdf.org/about-ocd/ocd-treatment-guide/erp/
kids.iocdf.org/professionals/md/treatment/

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